

STATE OF NEW YORK

7879--B

2025-2026 Regular Sessions

IN SENATE

May 13, 2025

Introduced by Sens. SALAZAR, ADDABBO, BAILEY, BORRELLO, BRISPORT, BROUK, CLEARE, COMRIE, COONEY, GOUNARDES, HARCKHAM, HINCHEY, JACKSON, LIU, MAY, MYRIE, PERSAUD, RAMOS, RIVERA, C. RYAN, SANDERS, SCARCELLA-SPANTON, SEPULVEDA, SERRANO, STAVISKY, WEBB, WEIK -- read twice and ordered printed, and when printed to be committed to the Committee on Women's Issues -- recommitted to the Committee on Women's Issues in accordance with Senate Rule 6, sec. 8 -- committee discharged, bill amended, ordered reprinted as amended and recommitted to said committee -- committee discharged, bill amended, ordered reprinted as amended and recommitted to said committee

AN ACT to amend the public health law, in relation to the duty to inform certain patients about the risks associated with cesarean section for patients undergoing planned and unplanned primary cesarean sections

The People of the State of New York, represented in Senate and Assembly, do enact as follows:

1 Section 1. The public health law is amended by adding a new section
2 2500-n to read as follows:

3 § 2500-n. Duty of providers of primary cesarean section services to
4 inform. 1. The commissioner shall require that every health care provid-
5 er, licensed pursuant to title eight of the education law and acting
6 within such health care provider's scope of practice and attending a
7 pregnant person, to provide written communication to each pregnant
8 person for whom a primary cesarean section delivery, defined as first
9 lifetime delivery via cesarean section, is planned.

10 2. In the event that the primary cesarean section is not planned
11 prenatally, the commissioner shall require that the health care provider
12 who performed the cesarean section provide written communication to each
13 person who delivered via primary cesarean section the reason for the
14 unplanned cesarean section after the delivery.

EXPLANATION--Matter in italics (underscored) is new; matter in brackets
[-] is old law to be omitted.

LBD00513-04-6

1 3. The provider shall provide communication to the patient with a
2 planned cesarean section. Such communication shall include, but not be
3 limited to the following information:

4 "Cesarean birth can be life-saving for the fetus, the birthing parent,
5 or both in some cases. However, in the absence of maternal or fetal
6 indications for cesarean delivery, a vaginal birth is preferable. Vagi-
7 nal delivery is associated with shorter recovery times, reduced risk of
8 infection, decreased maternal morbidity, and decreased risk of compli-
9 cations in future pregnancies. Potential injuries to the birthing
10 parent associated with cesarean birth include, but are not limited to,
11 heavy blood loss that results in hysterectomy or a blood transfusion,
12 ruptured uterus, injury to other organs including the bladder, and other
13 complications from a major surgery. Additionally, the risk of develop-
14 ing placenta previa or accreta is higher after cesarean delivery than
15 vaginal birth. Some providers may recommend a cesarean due to maternal
16 age, size of the fetus, or breech presentation, although this is widely
17 debated among providers. After a cesarean delivery, future vaginal
18 deliveries may result in uterine rupture. Because of this, cesarean
19 delivery may be recommended in the future. However, vaginal birth after
20 cesarean (VBAC) may be possible, depending upon your health character-
21 istics. Speak to your health care provider about your options and any
22 questions you may have."

23 4. The provider shall provide written communication to the patient
24 with an unplanned cesarean section that shall include, but not be limit-
25 ed to, the written communication required pursuant to subdivision two of
26 this section and the following information:

27 "Your most recent delivery was via cesarean section. Cesarean delivery
28 can be life-saving for the fetus, the birthing parent, or both in some
29 cases. After a cesarean delivery, future vaginal deliveries may result
30 in uterine rupture. Because of this, cesarean delivery may be recom-
31 ended in the future. However, vaginal birth after cesarean (VBAC) may
32 be possible, depending upon your health characteristics. Vaginal birth
33 after cesarean (VBAC) requires no abdominal surgery, shorter recovery
34 periods, lower risk of infection and may prevent certain health problems
35 linked to multiple cesarean deliveries. The risk of developing placenta
36 previa or accreta increases with each subsequent cesarean delivery. Not
37 all providers and hospitals perform VBACs. Speak to your health care
38 provider about your options and any questions you may have."

39 § 2. This act shall take effect on the one hundred eightieth day after
40 it shall have become a law. Effective immediately, the addition, amend-
41 ment and/or repeal of any rule or regulation necessary for the implemen-
42 tation of this act on its effective date are authorized to be made and
43 completed on or before such effective date.